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SITUATION REPORT · SITREP No. 059

MVE — 17th EPIDEMIC · DRC

17th Ebola Virus Disease epidemic in Democratic Republic of the Congo

Bundibugyo Ebolavirus · Ituri, North Kivu, South Kivu, Haut-Uélé and Tshopo Provinces · **Daily Edition** — Epidemiological Summary, Entry Points and Data Visualization

Reporting date: July 12, 2026 | **Publication date:** July 13, 2026 | **Daily Edition** | **Health Zones :** 42/140 | **Declared** May 15, 2026

KEY INDICATORS — MUL CURRENT AS OF JULY 12, 2026

 CONFIRMED CASES — 5 PROVINCES 1963 * +31 new cases (24 hours)	 CONFIRMED DEATHS · LEATHITIS 719 · 36.6%** +10 deaths reported (7 of which were community-acquired)	 SUSPECTED CASES OF THE DAY 268 including 112 community deaths
 PATIENTS IN ISOLATION 736 Occupancy 88.3% · North Kivu saturated	 CURED — CUMULATIVE 333 +15 recovered (Ituri 13 · N-Kivu 2)	 CONTACT TRACKING — 2 PROV. 81.5% Ituri 79.4% · North Kivu 90.5%

* Consolidated cumulative total (DHIS2). On July 12: +31 new confirmed cases and +10 reported deaths; the cumulative total also includes the revised Wamba health zone (Haut-Uélé), now reporting 7 confirmed cases, all deceased (from Nia-Nia). ** Overall case fatality rate calculated on consolidated totals (718 / 1,963); 34.3% in Ituri and 56.5% in North Kivu. Recoveries: provincial cumulative total (Ituri 317, North Kivu 27), national consolidation in progress.

1. Epidemiological Highlights

- **31 new confirmed cases (Ituri 27, North Kivu 4) and 10 deaths reported, including 7 community deaths in Ituri** — Bunia 3, Lita 2, Rwampara 1, Fataki 1 — and 3 in North Kivu (Katwa, Butembo).
- **Ituri accounts for 27 of the 31 new cases and all of the day's community deaths** ; transmission remains supported in the Djugu–Irumu corridor (Bunia, Lita, Nizi, Rwampara).
- Tshopo (Kisangani — Makiso-Kisangani, Mangobo, and Lubunga health zones; 4 cases / 2 deaths) and **Haut-Uélé** (Wamba: 7 confirmed cases, all deceased, originating from Nia-Nia) remain linked to the Ituri epicenter; the Kisangani cases all originate from the Nia-Nia health zone (Ituri). **No new health zones were reported on July 12**, the national total remaining at 42 out of 140.
- **South Kivu — 47 consecutive days without a new confirmed case** (as of July 12, 2026), more than twice the maximum incubation period: the province maintains residual surveillance with a view to declaring the end of the epidemic.

- **Fifteen (15) patients declared recovered** (Ituri 13, North Kivu 2 at the Beni ETC). Deployment of **3 additional EDS teams to Nizi** (6 in total) in response to community mortality.
- **Entry points/checkpoints:** 71/71 PoCs activated and PoEs reinforced (100%); **144,094 travelers screened** (Ituri + North Kivu); 7 alerts notified, 2 live alerts validated and referred, and 4 bodies intercepted. Activation of the Makoko 2 PoC (Mambasa) and partial opening of the Ruzizi I and II PoEs (South Kivu).
- **Cumulative:** 1,963 confirmed cases and 718 deaths — case fatality rate of **36.6%** — in 42 out of 140 health zones (5 provinces); Progress since SitRep N°058: +31 cases and +10 deaths.

2. Descriptive Epidemiological Analysis

2.1 — Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality in affected health zones	New cases (24 h)
Ituri	1,772	608	34.3% 26/36 (72.2%)	27
North Kivu	177	100	56.5% 11 / 34 (32.4%)	4
South Kivu	3	1	33.3% 1/34 (2.9%) 100.0%	0
Haut-Uélé	7	7	1/13 (7.7%)	0
Tshopo	4	3	75.0% 3/23 (13.0%)	0
Total	1963	718	36.6% 42 out of 140	31

At the end of July 12, 2026, no new health zones were reported; the national total remains at **42 out of 140 health zones**, spread across 5 provinces (Ituri 26/36, North Kivu 11/34, Tshopo 3/23, including Lubunga). As of July 12, 2026, South Kivu had gone **47 consecutive days without a new confirmed case** (more than twice the maximum incubation period), paving the way for declaring the end of the epidemic in the province, subject to continued epidemiological vigilance.

Provincial dynamics and case concentration

Approximately **90.5% of cases (n=1,772)** and 85.4% of deaths (n=608) are concentrated in Ituri, which remains the epicenter of the epidemic (crude case fatality rate 34.3%). The areas of Bunia, Rwampara, Mongbwalu, Nizi, and Nyankunde account for the majority of cases in the province. On July 12, **Bunia (11 cases), Lita (6), and Nizi (5)** accounted for the highest number of new cases, and all seven deaths that day occurred in the community. **Mongbwalu has the highest severity rate (52.0%, or 171 deaths out of 329 cases)**, primarily due to the high proportion of community deaths.

Geographic extension — Haut-Uélé and Tshopo

Tshopo province has three affected health zones— **Makiso-Kisangani, Mangobo, and Lubunga** (Kisangani; provincial total: 4 cases, 2 deaths)—along with **Haut-Uélé** province (Wamba health zone: 7 cases, 7 deaths, all from Nia-Nia). No new cases were reported there on July 12. All these cases are linked to the epicenter in Ituri (Nia-Nia/PK51) and do not constitute local transmission. The city of **Kisangani (approximately 1.6 million inhabitants)** nevertheless increases the risk of urban, river, and airborne spread. Interventions there have been strengthened—surveillance, teams for dignified and safe burials, and isolation and care capacities.

2.2 — Distribution by health zone as of 12/07/2026

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)	
	Case (n)	Deaths (n)	Lethality	New case	Death notification.
Ituri	1,772	608	34.3%	27	7
Bunia	503	156	31.0%	11	3
Rwampara	384	89	23.2%	3	1
Mongbwalu	329	171	52.0%		
Nizi	110	36	32.7%	5	0
Nyankunde	97	23	23.7%		
Mangala	50	29	58.0%		
Lita	45	19	42.2%	6	2
Nia-Nia	35	13	37.1%		
Bamboo	31	10	32.3%		
Komanda	25	8	32.0%		
Tchomia	19	4	21.1%		
Unidentified	17	0	0.0%		
Kilo	9	1	11.1%		
Mandima	8	3	37.5%		
Logo	7	0	0.0%		
Aungba	6	2	33.3%		
Damascus	5	0	0.0%		
Mambasa	5	4	80.0%		
Rimba	4	0	0.0%		
Fataki	4	1	25.0%	1	1
Drodro	4	3	75.0%	1	0
Aru	3	1	33.3%		
Kambala	2	2	100.0%		
Gety	1	0	0.0%		
Lolwa	1	1	100.0%		
Boga	1	1	100.0%		
Ariwara	1	1	100.0%		
North Kivu	177	100	56.5%	4	3
Katwa	71	47	66.2%	2	2
Butembo	44	20	45.5%	2	1
Blessed	30	20	66.7%		
Musienene	10	2	20.0%		
Kyondo	7	3	42.9%		
Oicha	3	2	66.7%		
Masereka	3	0	0.0%		
Kalunguta	2	1	50.0%		
Goma	1	0	0.0%		
Vuhovi	1	1	100.0%		
Mabalako	1	0	0.0%		
South Kivu	3	1	33.3%	—	—
Miti-Murhesa	3	1	33.3%		
Haut-Uélé	7	7	100.0%	—	—

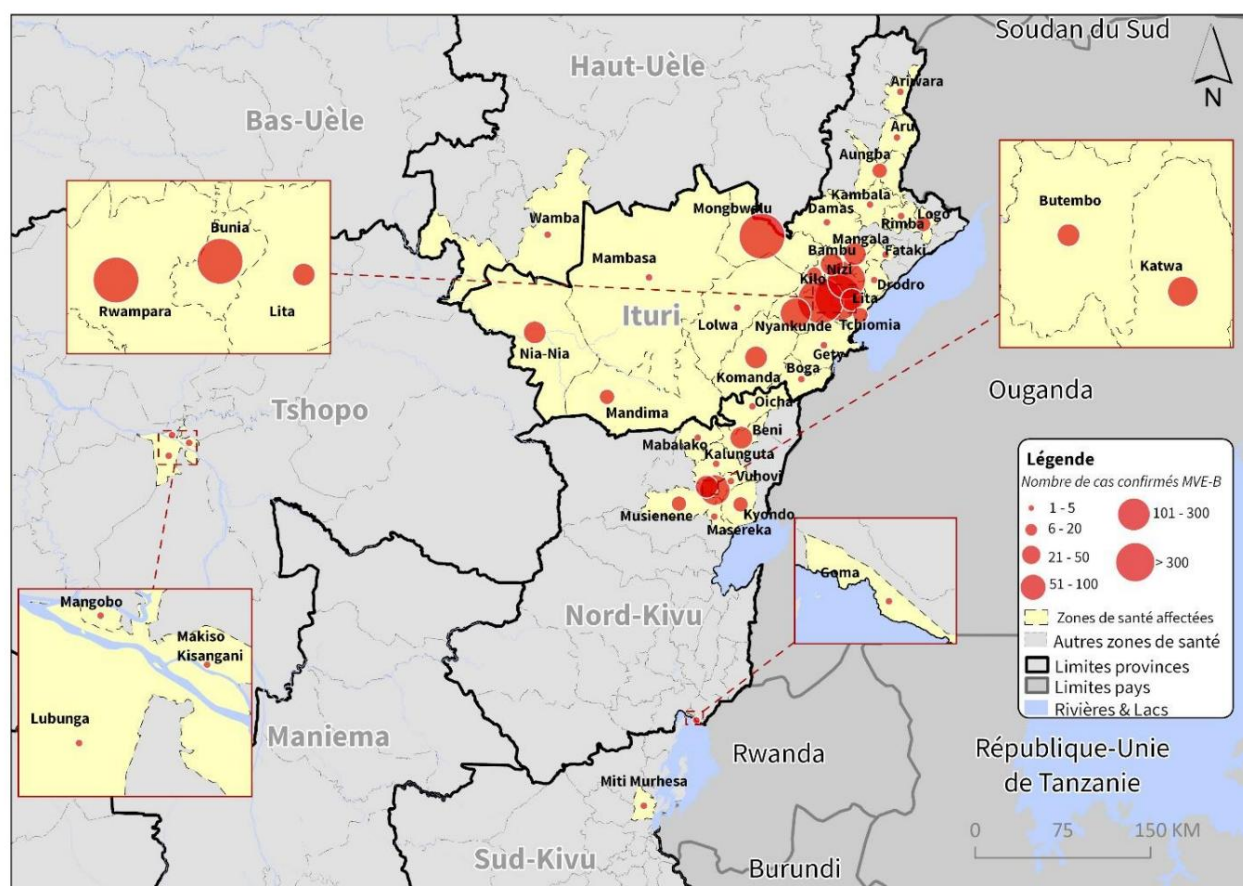
Province / Health Zone	Cumulative number			Current situation (24 hours)	
	Case (n)	Deaths (n)	Lethality	New case	Death notification.
Wamba	7	7	100.0%		
Tshopo	4	3	75.0%	—	—
Makiso-Kisangani	2	2	100.0%		
Mangobo	1	0	0.0%		
Lubunga	1	0	0.0%		
Total	1963	718	36.6%	31	10

Note — The "Daily Situation" column shows the new confirmed cases and deaths reported on July 12 (cells marked "—" or empty indicate no activity that day). Ituri: the cumulative distribution by health zone corresponds to the last consolidated distribution (July 10); the provincial total (1,772 cases / 608 deaths) is the cumulative total as of July 12, the difference from the sum of the health zones reflecting the cases and deaths accumulated since that distribution and the ongoing DHIS2 reclassifications. The 7 deaths reported in Ituri on July 12 all occurred in the community; in North Kivu, 3 deaths (Katwa 2, Butembo 1). Haut-Uélé: the Wamba health zone is revised to 7 confirmed cases, all deceased (from Nia-Nia).

2.3 — Mapping of confirmed cases

The spatial distribution confirms the concentration of cases in the **Djugu-Irumbu** corridor (Ituri) — Bunia, Rwampara, Mongbwatu, Nizi — with an extension towards North Kivu (Beni, Butembo, Katwa). The map below shows the health zones that have reported confirmed cases nationwide.

FIGURE 1 — CONFIRMED CASES BY HEALTH ZONES

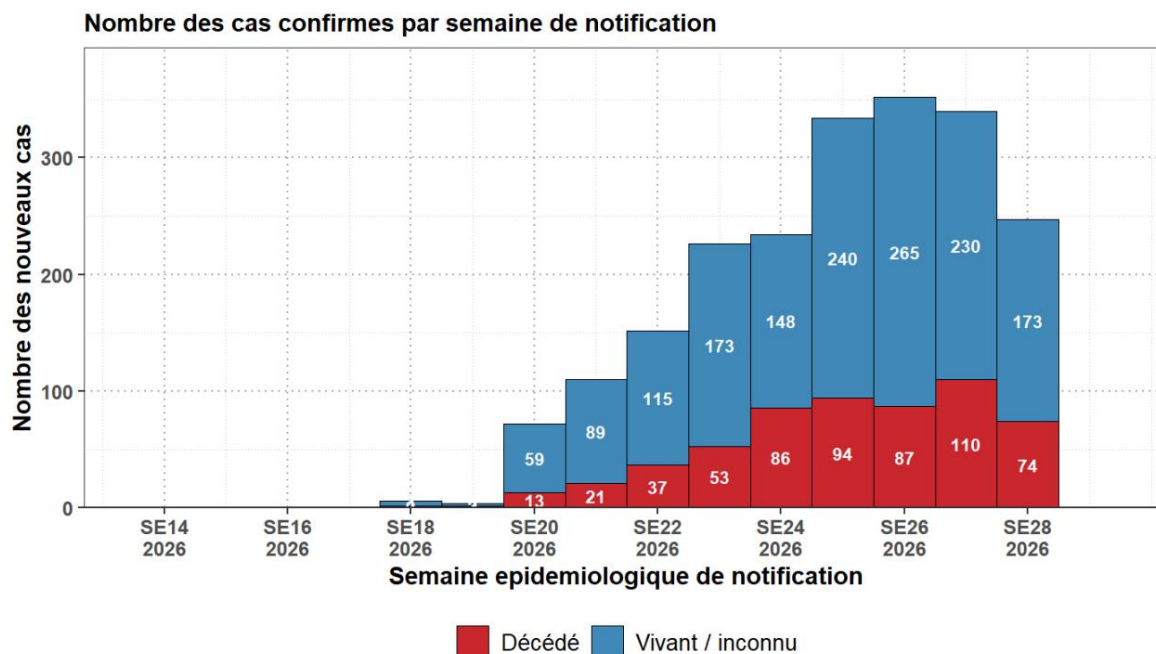


2.4 — Temporal analysis — epidemic curves

We are seeing an increasing number of confirmed cases from one week to the next, reflecting ongoing community transmission. Epidemiological weeks **S25** and **S26** confirm this continued growth, with over 300 cases.

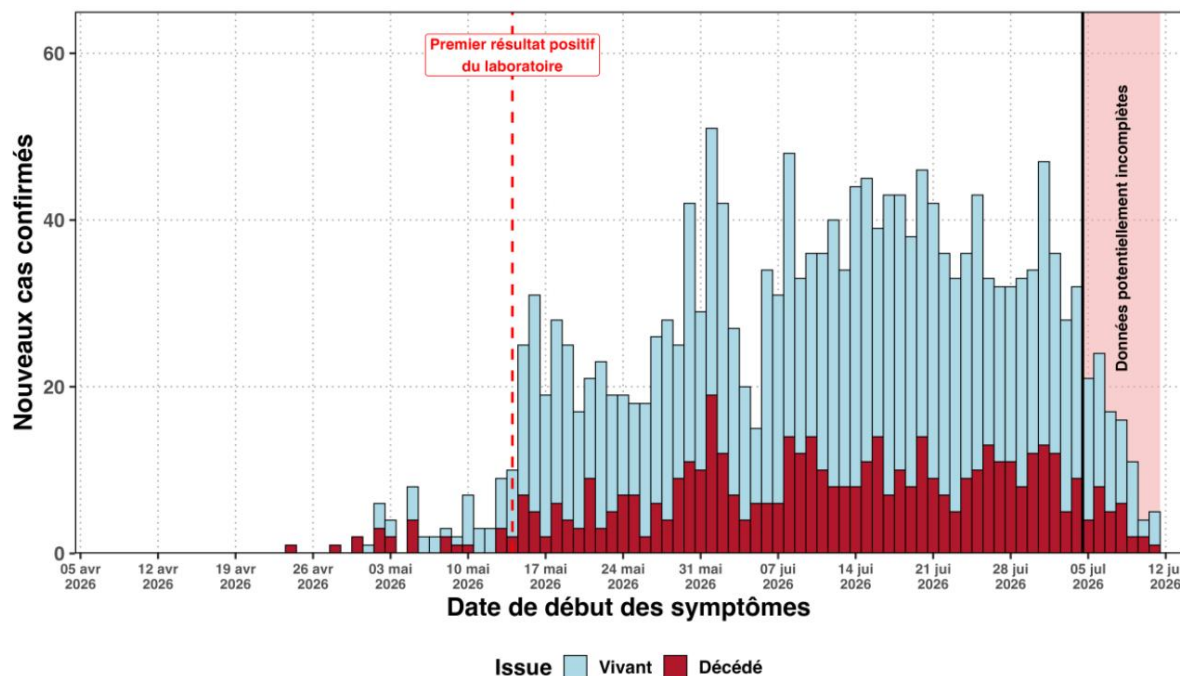
confirmed per week. Two complementary views are presented: the **curve by week of notification** (COUSP) and the **curve by date of onset of symptoms** (linear list DHIS2).

FIGURE 2 — EPIDEMIC CURVE PER NOTIFICATION WEEK (COUSP)



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses : cas confirmés avec statut vital et date de notification renseignés, n = 2 077.

FIGURE 3 — EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)



2.5 — Demographic characteristics

The age pyramid shows a predominance of **working-age adults** : the 20–29 and 30–39 age groups account for the majority of confirmed cases, with **more women than men in the 20–29 age group**. Investigations into these differences are ongoing; they could reflect differences in risk exposure (for example

among caregivers). Children under 10 and the elderly are proportionally less affected, a profile consistent with transmission linked to community, professional and care activities.

FIGURE 4 — CONFIRMED CASES BY SEX AND AGE GROUP

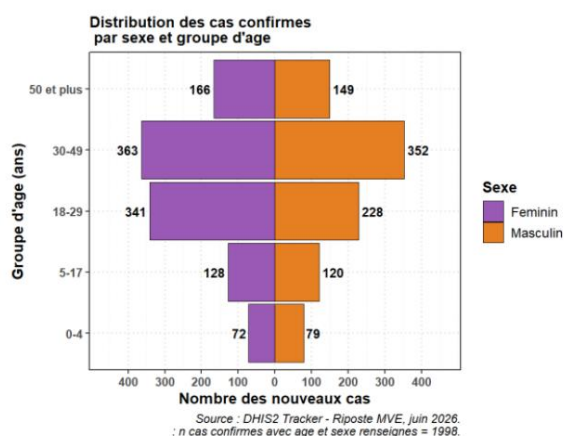
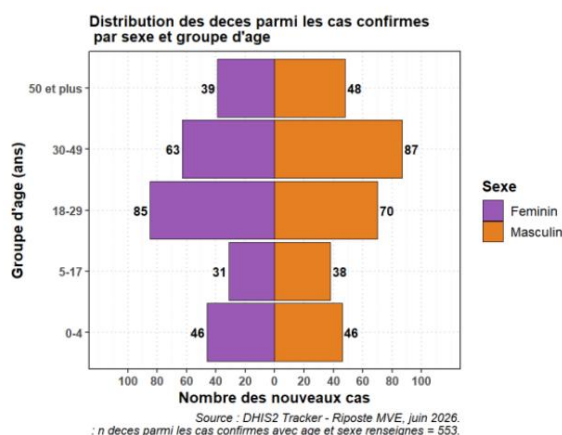


FIGURE 5 — DEATHS BY SEX AND AGE GROUP (AMONG THE CONFIRMED CASES)



3 Monitoring, testing and reporting

3.1 — Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS (24 H)

Indicator	Ituri	North Kivu	South Kivu	Haut-Uélé	Total
Report (D-1)	17	29	0	0	46
New alerts received	408	665	5	32	1110
Total alerts for the day	425	694	5	32	1,156
Alert investigation rate (24 h)	88.0%	95.8%	80.0%	96.9%	92.9%
Alerts confirmed — live	118	23	4	11	156
Alerts confirmed — deceased (community)	56	39	0	17	112
Total suspected cases for today	174	62	4	28	268

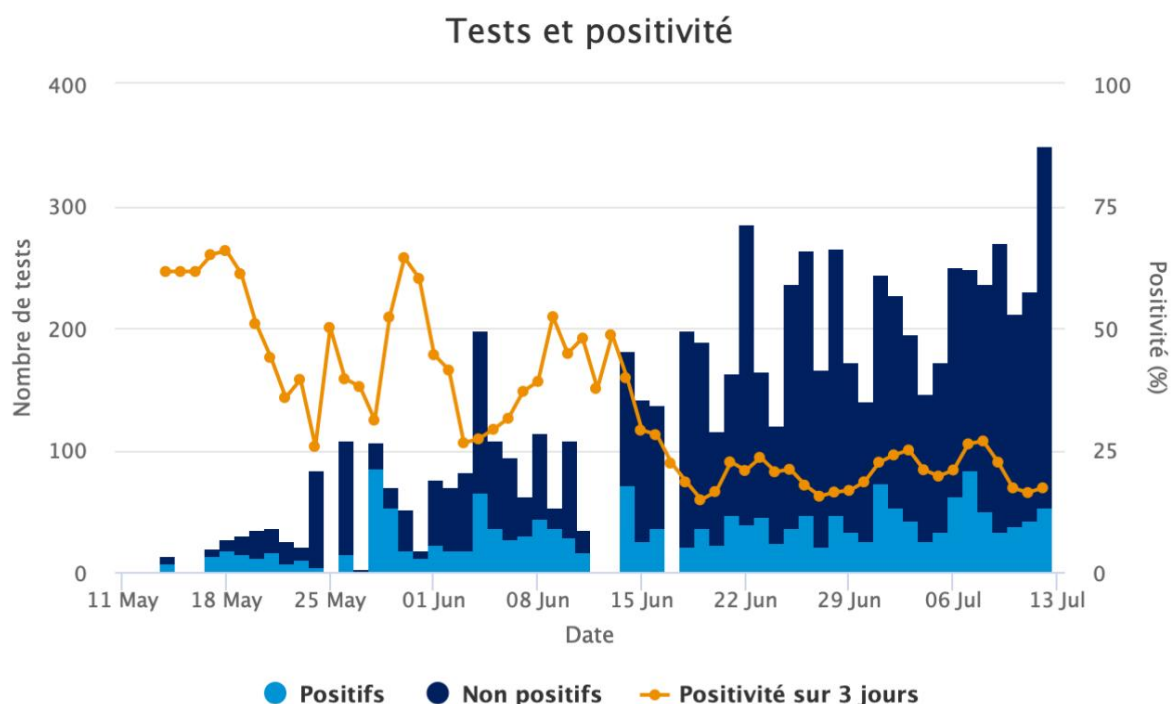
1,156 alerts were recorded in the four active provinces, including 1,110 new ones. **268 suspected cases were confirmed**, including 156 (58.2%) living cases and 112 (41.8%) community deaths under investigation. North Kivu accounts for the majority of alerts (694, or 60.0%), followed by Ituri (425, 36.8%), Haut-Uélé (32), and South Kivu (5). In Ituri, 83 of the 118 living suspected cases have been isolated (70.3%). The alert rate remains proportionally low in Ituri, as evidenced by a significantly higher positivity rate than in the rest of the country. North Kivu.

3.2 — Laboratory: tests and positivity

- **Ituri:** 203 samples analyzed (189 blood, 14 swab) — **27 positive** (positivity 13.3%): Bunia 11, Lita 6, Nizi 5, Rwampara 3, Fataki 1, Drodoro 1.
- **North Kivu:** 99 samples analyzed — 6 positive (positivity 6.1%), concentrated in Butembo (Beni 0).
- **South Kivu:** 8 samples received today (cumulative 231 analyzed, positivity 1.3%); no new positive results.

In Ituri, 203 samples were analyzed with a positivity rate of **13.3%** — down despite sustained volume; North Kivu maintains a low yield (6.1%) and the backlog of samples remains to be cleared.

FIGURE 6 — TESTS AND POSITIVITY (COUSP SITUATION REPORT)



N = 7,390 tests. Source : rapport de situation COUSP.

3.3 — Contact tracing and transmission chains

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	9,866	7,830	79.4%
North Kivu	2,377	2,152	90.5%
Total	12,370	10,156	82.1%

405 new contacts were listed in Ituri and 164 in North Kivu; 8 contacts became suspected cases in Ituri (12 in North Kivu), and 364 completed their 21-day follow-up. Six zones (Gethy, Lolwa, Mandima, Logo, Rimba, Kambala) did not submit their reports in Ituri. In South Kivu, 226 contacts are under follow-up, including 2 who tested positive. By hotspot zone, follow-up varies considerably: Bunia 96.9%, Nizi 92.6%, Mongbwalu 89.0%, Rwampara 85.2%, but drops significantly in Nyankunde (28.9%), Nia-Nia (36.2%), and Kilo (44.0%).

As of July 12, **none of the 27 new confirmed cases in Ituri were previously listed contacts (0%)** : almost all cases occurred outside of known transmission chains. Cumulatively, the proportion of cases linked to a Recorded contact remains low (̄ 15–20%). Among documented contacts, neighborhood **and family** relationships are the most common. predominate.

Analysis by circumstances shows that exposure at home (household) mainly involves family ties, while contacts in **hospital, funeral and professional** settings have a high proportion of "unknown" relationships — profiles at risk of transmission outside the household.

3.4 — Management of suspected cases

TABLE 5 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI)

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	538	173	16	727
Total admissions (24 hours)	85	12	24	121
Exits — deceased	15	2	1	18

Indicator	Ituri	North Kivu	South Kivu	Together
Exits — non-cases	33	11	21	65
Exits — cured	13	2	0	15
Exits — escapees	9	0	0	9
Discharges — transferred to the HGR	0	0	1	1
Total outflows (24 h)	70	15	23	108
Patients in isolation (end of day)	549	170	17	736
including those confirmed	229	17	0	246
including suspects	320	153	17	490
Occupancy rate	82.1%	117.2%	nd	88.3%

The situation remains critically strained in North Kivu (117.2%), with a severe shortage of isolation beds; Ituri (82.1%) remains under pressure despite the opening of extensions (Bunia General Referral Hospital Treatment Center, Kingozi camp Treatment Center). A total of 736 patients are in isolation (246 confirmed, 490 suspected). Two suspected cases were released from isolation in Ituri before receiving their test results.

4. Entry points and control points (PoE/PoC)

The 71 points of control (PoCs) and points of entry (PoEs) in the outbreak zones remain fully operational and reinforced (Ituri 57, North Kivu 14). As of July 12, 144,094 travelers were screened in Ituri and North Kivu (95.4% screening, 98.1% awareness campaign). Seven (7) alerts were notified—2 live alerts were validated and referred to the CT/CTE, and 4 bodies were intercepted.

4.1 — Alerts at crossing points (24 h)

Ituri — 5 alerts (3 alive, 2 dead)

Two confirmed live alerts were isolated and referred: at the Nyamusasi PoC, a 26-year-old man (TchomiaïKasenyi, fever), transferred to the CT; at the Banguba PoC, a 32-year-old woman (PK51/Nia-Nia, fever, vomiting, headache), transferred to the CTE. One alert (Marabo PoC, 16-year-old boy, BeniïHoho) was invalidated after reassessment. Two bodies were intercepted — Nyori PoC (swabé) and Watsa Road PoC (awaiting EDS). Note: the activation of Makoko 2 PoC (Mambasa Health Zone).

North Kivu — 2 alerts (2 bodies)

Two bodies were intercepted and transported for exhumation: at the Mukulia PoC, a 60-year-old man (BiakatoïButembo), who was taken to the morgue at the Beni General Referral Hospital; and at the Mususa PoC (Musienene Health Zone), a 79-year-old woman (ButemboïMbau). No confirmed sightings of living bodies were reported in North Kivu today.

South Kivu — 5 alerts (all alive) · preparedness measures

Five alerts notified to the Kazingo and Ihusi PoCs — 3 validated and placed at the UOB transit center, 2 invalidated. Partial opening of the Ruzizi I and Ruzizi II PoEs. During the period from June 22 to July 12, 49,187 travelers passed through the PoEs (including 1,317 in international traffic) and 637,319 through the PoCs; 100% of the province's PoCs/PoEs remain activated (68,082 people checked as of July 12).

4.2 — Monitoring indicators for PoE/PoC

TABLE 6 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC), AS OF JULY 12, 2026

Indicators	Ituri	North Kivu	Overall
Completeness of daily reports	100% (57/57)	100% (14/14)	100% (71/71)
Proportion of PoC enabled and PoE enhanced	100% (57/57)	100% (14/14)	100% (71/71)
Number of people who have switched to PoE/PoC	90,389	53,705	144,094

Indicators	Ituri	North Kivu	Overall
% of travelers screened	93.9%	98.0%	95.4%
% of travelers who washed their hands	93.0%	98.0%	94.9%
% of travelers made aware	98.2%	98.0%	98.1%
Number of alerts notified	5	2	7
Number of validated live alerts	2	0	2
Number of bodies recovered today	2	2	4
Number of problematic contacts intercepted today	0	0	0
Percentage of validated live alerts referred to the CT/CTE	100.0%	50	100.0%
% of lifeless bodies swabed	50.0%	0.0%	25.0%
Number of positive results of referred suspected cases	0	0	0

Consolidated indicators for the PoE/PoC pillar as of July 12, 2026 (Ituri + North Kivu). Bodies swabé: 1 out of 2 in Ituri (Nyori swabé, Watsa Road pending); both bodies in North Kivu (Mukulia, Mususa) transported for sampling. South Kivu (100% of PoC/PoE activated, 68,082 people screened) is being monitored separately (see 4.1).

5 Pillar-based response activities — summary of July 12

COORDINATION Meetings with partners (PTF) and field supervisions. Health Minister's plea to Ituri leaders: announcement of 10 ambulances and health/bank cards to secure payment of agents; commitment from provincial deputies.	PCI / WASH Ituri: 42/67 transient households decontaminated in 48 hours; 12/12 ESS (Emergency Situations) of confirmed cases decontaminated; 924/1,640 service providers trained. North Kivu: decontamination of the morgue and ESS (Beni, Katwa, Kalunguta); 18 ESS assessed (score < 50%); 15 service providers trained (Beni).
Dignified and secure burials (DSM) Ituri: deployment of 3 additional teams to Nizi (6 in total); completion of EDS and swabs; shortage of inputs reported in Nizi. North Kivu: 36 death alerts, 35 bodies recovered and secured, 29 EDS completed.	HOLISTIC CARE (HCC) 15 new recoveries (Ituri 13, North Kivu 2); 8 ambulance trips (3 operational, 2 out of service); 987 hot meals served; 2 suspected cases released from isolation before results (Ituri). Mental health (SMSPS): see Table 8.
COMMUNICATION (RCCE / CREC) Ituri: Ceremony marking the release of the healed; commitment of provincial deputies; 1,265 people affected through home visits. North Kivu: community dialogues (Katwa, Beni, Butembo, Mabalako); 7,836 households visited; 25,398 people reached door-to-door; 30/34 radio stations broadcasting.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) Development of the response plan; mapping of partners offering free care (18/36 health zones in Ituri); monitoring of frontline staff assigned (see Table 9).
RESEARCH, INNOVATION & VACCINATION EBO-PEP clinical trials (ALIMA) in preparation; inter-partner meeting on 13/07 (INRB, ALIMA, WHO).	LOGISTICS North Kivu: deployment of teams; delivery of IPC kits and medicines (Mambangu Military Hospital); repair of mobile vehicle 103 (Mutsora). Ituri: support for the transport of supplies and deployment of key personnel.
SECURITY No new major incidents reported on July 12; heightened vigilance around PoE/PoC and support structures; coordination maintained for access to risk areas.	NUTRITION South Kivu: distribution of hot rations to cases in isolation (Fomulac, Lwiro). Ituri: 16 specific diets and hot meals served (nursery, caregivers, frontline staff).
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) South Kivu: transmission of the SITREP system to the PEAS network; awareness-raising on EAS among community health workers and the community; 50 service providers briefed and signatories of codes of conduct; promotion of zero tolerance and the toll-free number (495555 / 151). Ituri: a total of 2,250 people briefed, coverage extended to 5 health zones.	

y Mental health and psychosocial support (MHPSS)

TABLE 8 — KEY INDICATORS OF MENTAL HEALTH AND PSYCHOSOCIAL SUPPORT (MHPSS) ACTIVITIES, JULY 12, 2026

SMSPS Key Indicators	Ituri	North Kivu	South Kivu
New confirmed cases taken into care (SMSPS)	6	4	0
Confirmed cases are being monitored	101	9	0
New suspected cases	38	65	7
Suspected cases are being monitored	93	66	20
Laboratory results announced (including positive ones)	21 (5)	35 (4)	2 (0)
Children separated — nursery / community	9/0	1 / —	1/22
CTE support staff	69	44	15
Frontline Personnel (PPL)	20	35	0
Household/Community Members	24	48	0
EDS supported (SMSPS)	5	6	0

y Frontline Personnel (PPL) assigned

TABLE 9 — FRONTLINE STAFF (FLS) INFECTED WITH EVD (CUMULATIVE AS OF JULY 12, 2026, ITURI)

Health Zone	Confirmed cases	Healed	In hospital	Death
Bunia	35	16	5	14
Rwampara	32	23	2	7
Mongbwalu	16	5	3	8
Nyankunde	14	11	1	2
Nia-Nia	5	0	5	0
Nizi	3	0	0	3
Mangala	3	1	1	1
Others (Kilo, Bambu, Lita)	6	2	3	1
Total — case fatality rate 31.6%	114	58	20	36

PPL = healthcare workers and frontline providers engaged in the response. Source: Continuity of Essential Services (CSSE) pillar, July 12, 2026. The high infection rate among staff (case fatality rate 31.6%) underscores the ongoing need for IPC and protective equipment.

6. Main challenges, impacts and actions required

Challenge No.	Impact	Actions required
1	Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Delayed confirmation, underestimation of cases, high community mortality
2	Insufficient capacity of CTE/CT facilities; saturation in North Kivu (117.2%)	Admission delays, risk of nosocomial transmission
3	Follow-up of contacts under the target (81.5% vs 95%) and heterogeneous (Nyankunde 28.9%, Nia-Nia 36.2%)	Uninterrupted transmission chains
4	Limited response and investigation of alerts in Ituri; inconsistencies in the ZS / DHIS2	Transmission not detected, silent expansion
5	None of the new cases linked to non-communicable transmission	

Intensify CREC, involve religious and community leaders, secure the teams, strengthen EDS teams (done: +3 in Nizi)

Accelerate the expansion of CTEs, deploy temporary structures, open more transit centers

Train, deploy and motivate community liaisons (RECOs); enhanced supervision

Strengthen community surveillance; harmonize ZS/ DHIS2 databases; add investigators

In-depth investigation, active research

Challenge No.		Impact	Actions required
	Contact listed (0% on 12/07)	mastered	expansion of the listing
6	Insufficient ambulances and isolation facilities	Delays in transfer and isolation	Preposition ambulances (including those at PoE/PoC), deploy CTs in each ZS
7	Sample backlog and diagnostics late in North Kivu	Delayed confirmation, high lethality (56.5%)	Laboratory catch-up plan, strengthening of diagnostic capacities
8	Infected frontline staff (114 cases, 36 deaths, case fatality rate 31.6%)	Loss of capacity and nosocomial risk	Strengthen PCI, equip with PPE and PCI/EHA kits
9	Insecurity and limited access (Nia-Nia, CODECO/ADF)	Restriction of operations	Strengthen security coordination and humanitarian access
10	High population mobility and urban sprawl (Tshopo/Kisangani)	Risk of urban, riverine and cross-border spread	Strengthen monitoring at PoE/PoC locations and cross-border coordination
11	Insufficient resources: mobility and payment of service providers	Demotivation of the players, inefficiency	Mobilize resources, regularize payments
12	Continuity of essential services compromised; insufficient MEG and nutritional inputs	Increase in indirect (non-Ebola) mortality	Make free healthcare a reality; supply essential medicines and nutritional inputs

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